

**PRODUCT**

MATERNALINK FEATURE ADDENDUM (8 September 2026)

Vytalix company-build documentation · reference
MATERNALINK_FEATURE_ADDENDUM

Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS_BASE.

PREPARED FOR

The Founder, Vytalix

DATE

8 September 2026

VERSION

1.0 · Confidential draft

Contents

Executive view	1
Feature map	2
Intended-use statement (draft for clinical and legal review)	3
What must change in the prototype before reuse	4
Priorities / Risks / Next actions	5

Maps every feature the founder described to a version, a regulatory posture and the evidence needed before it is sold or claimed. Read with /docs/04_MATERNALINK_PRODUCT_STRATEGY.md , /product/MATERNALINK_PRD.md and /docs/21_MATERNALINK_PRICING.md . Status labels: PROPOSED unless stated.

Executive view

1. Most of the founder's feature list fits v1 as **non-diagnostic engagement and coordination**: daily questionnaire, symptom log, partner engagement, e-care database, appointment efficiency, voice diaries as storage.
2. Three features carry **medical-device exposure** and need a regulatory opinion before they are configured or claimed: triage prioritisation, glucose interpretation, and any wearable that measures physiology for clinical use.
3. **Voice tracing of baby movements** is an unvalidated research idea and must not appear in any sales material as a feature.
4. **Replace vs integrate** is a deployment mode, not two products: Core can be the working programme record where none exists; where a maternity record exists, MaternaLink integrates and never becomes the legal record.
5. The prototype already implements a risk engine with suggested clinical actions; that is the single biggest gap between "what exists" and "what can be claimed".

Feature map

Founder's feature	Product mapping	Version	Regulatory posture	Evidence before claim	Pricing code
E-care patient database ("Maternity watch" record)	Core registry: mother + newborn, timeline, notes, exports; provider-configurable fields	v1	Non-diagnostic record support; data-protection controls; never the legal record where one exists	DPIA, DSPT/Cyber Essentials, audit log tests	ML-CORE
Given at the week-16 midwife appointment; returned after postnatal discharge	Enrolment and discharge workflow : consent, onboarding, device issue/return log, offboarding and data retention rules	v1 (software) · device later	Software: none beyond data protection. Device: see Watch row	Onboarding time and completion rates in pilot	ML-CORE (+ML-WATCH)
Midwife better explanation	Clinician-signed-off education library, delivered in-app and by SMS/WhatsApp, multilingual roadmap	v1	Educational content; governed by a content sign-off process	Comprehension and engagement measures	ML-ENGAGE
Decrease the appointment time	Pre-appointment check-in and symptom summary for the clinician; structured handover	v1	Non-diagnostic summary; clinician reviews raw entries	Time-and-motion study in the pilot (do not claim minutes saved before measured)	ML-CORE

Founder's feature	Product mapping	Version	Regulatory posture	Evidence before claim	Pricing code
Maternity Triage / Maternity A&E	Triage intake module: structured presenting-complaint capture, contact and outcome logging, prioritisation rules configured and owned by the site's clinical governance	v2 (TEST first)	Likely clinical decision support if Vytalix ships rules; safe posture = rule editor without default rules; obtain MHRA classification opinion	Regulatory opinion; one-site service evaluation	ML-TRIAGE
Daily questionnaire (like a period app)	Daily check-in with configurable questions; streaks and reminders; answers visible to the care team	v1	Non-diagnostic; every concerning answer routes to a human within a defined SLA	Engagement rates; escalation handling audit	ML-ENGAGE
Quizzes and section for partners	Partner account linked with the woman's consent; education quizzes; prompts (appointment companion, warning signs to watch)	v1	Consent and safeguarding: the woman controls partner access and can revoke it silently (domestic-abuse safe design)	Safeguarding review; user testing with charities	ML-ENGAGE
Log symptoms: itching hands and feet, vomiting, etc.	Symptom log with plain-language prompts; information only: "itching of hands and feet in pregnancy should be reported to your midwife today" with one-tap contact; no diagnosis (no "cholestasis" label generated by software)	v1	Non-diagnostic if the software neither scores nor interprets; wording reviewed by clinicians	Content sign-off; hazard log entry per symptom	ML-ENGAGE
Women with diabetes: input blood-sugar levels	Self-reported glucose diary with clinician-set targets; trend view; threshold notification to the care team	v2 (DEFER)	Threshold alerting on physiological data edges toward device classification; obtain opinion before build	Regulatory opinion; clinician co-design	ML-GLUCOSE
Voice tracing of babies' movements	Not a feature. Research track: feasibility of movement-pattern capture requires a validated sensor, ethics approval, a clinical partner and a device pathway	v3 research	Medical device by intended purpose if it informs care	Ethics, protocol, data partnership, validation study	none
Voice diaries	Storage and playback attached to the timeline; optional transcription/translation roadmap	v1	Storage only; no analysis	Consent flows; retention policy	ML-VOICE

Founder's feature	Product mapping	Version	Regulatory posture	Evidence before claim	Pricing code
Maternity Watch (wearable)	Integration of a certified wearable for prompts and, where the device is certified for it, physiological readings synced to the record	v2/v3 (partner device)	The device carries its own UKCA/CE marking; MaternaLink displays data without interpreting it unless a pathway is in place	Supplier due diligence; device MDR status; DPIA update	ML-WATCH
Replacing hospital systems	Deployment mode A: MaternaLink Core as the programme's working record where none exists	v1	Data-protection and clinical-safety case for record-keeping; still not a certified EPR	Clinical-safety case (DCB0129/0160 equivalent)	ML-CORE + implementation
Integrating with hospital systems	Deployment mode B: interfaces to BadgerNet/K2/Euroking, DHIS2/HMIS, SSO	v1–v2	Standard integration; vendor agreements	Interface tests; vendor sign-off	ML-INTEGRATE

Intended-use statement (draft for clinical and legal review)

"MaternaLink is a care-coordination and communication platform that helps maternity programmes enrol women, keep in contact between appointments, record self-reported information and clinician notes, and route reported concerns to the responsible clinician. It does not diagnose, predict, triage or recommend treatment. Any prioritisation rules are configured, owned and reviewed by the deploying organisation's clinical governance."

What must change in the prototype before reuse

1. Remove or relabel the "suggested actions" output so that no clinical recommendation is generated by default; replace with "Programme escalation rule: [name] — contact the responsible clinician".
2. Remove identifiable photographs and real-looking test identities; use synthetic data.
3. Put the public URL behind authentication; confirm no real patient data was ever entered.
4. Rename to MaternaLink; add the Vytalix mark; align copy to this addendum.

Priorities / Risks / Next actions

Priorities: v1 = Core + Engage + Voice storage; Triage and Glucose behind a regulatory opinion; Watch as a partner device.

Risks: feature creep back into decision support; partner-access safeguarding failures; over-promising the watch.

Next actions: clinician review of the intended-use statement (Day 14); regulatory opinion scoped (Day 21); prototype changes 1–4 (Day 14).